

Name : Mr. GURMEET SINGH

Lab No. : 463577624

Ref By : DR ADEEB SINGH

Collected : 19/12/2024 3:32:00PM

A/c Status : P

 Collected at : Hexamed Healthcare Services LLP  
 25-270/6, Taluka Bahu, Main Bypass Road, NH1A,  
 Channi Himmat,  
 Jammu,

Age : 52 Years

Gender : Male

Reported : 21/12/2024 1:24:16PM

Report Status : Final

### Test Report

#### STONE ANALYSIS BY FTIR



Sample: Kidney Stones

Aggregate Weight: 0.201 gm

Composition : Calcium Oxalate Monohydrate 90%

Calcium Oxalate Dihydrate 10%

#### (Representative stone analysed)

Kidney stone (Nephrolithiasis) develops when urine becomes supersaturated with crystal forming substances such as Calcium, uric acid, oxalate etc.(resulting from dehydration or genetic predisposition) or due to decrease in urinary concentration of substances such as magnesium, citrate etc. that inhibit stone formation. Kidney stones tend to recur; approximately 50% of patients who form one stone form another in 10 years. Nephrolithiasis is considered to be a disease of affluence because it is more prevalent in conditions like Obesity, Hypertension & Diabetes. For patients with multiple kidney stones 24-hour urine chemistries

should be measured to identify abnormalities that predispose to kidney stones, which guides dietary and pharmacologic therapy for preventing stone recurrences.

### A Unit of Hexamed Healthcare Services LLP

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| STONE TYPE                              | FREQUENCY (%) | CAUSES                                                                                                                                                                                                                                                                                                                                       |
|-----------------------------------------|---------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Calcium oxalate / mix                   | 75            | <ul style="list-style-type: none"> <li>Hypercalciuria with Hypercalcemia (Hyperparathyroidism, Vitamin D excess, malignancy)</li> <li>Idiopathic hypercalciuria</li> <li>Hypocitraturia</li> <li>Alkaline urine, low urine volume</li> <li>High dose of vitamin C</li> <li>Metabolic syndrome</li> <li>Inflammatory bowel disease</li> </ul> |
| Calcium Phosphate                       | 5             | <ul style="list-style-type: none"> <li>Distal Renal Tubular acidosis</li> <li>Primary Hyperparathyroidism</li> <li>Chronic urinary tract infection</li> <li>Hypercalciuria and/or Hypophosphaturia</li> </ul>                                                                                                                                |
| Magnesium Ammonium Phosphate (Struvite) | 10-20         | Recurrent urinary tract infection                                                                                                                                                                                                                                                                                                            |
| Uric Acid                               | 5-10          | <ul style="list-style-type: none"> <li>Hyperuricosuria and /hyperuricosemia</li> <li>Metabolic syndrome (Diabetes, obesity, Hypertension)</li> <li>Acidic urine (pH&lt;5.5)</li> <li>Gout</li> </ul>                                                                                                                                         |
| Cystine                                 | 1             | Cystinuria (A genetic disorder)                                                                                                                                                                                                                                                                                                              |

### DIETARY RECOMMENDATIONS FOR PREVENTION OF RECURRENCE

#### General Dietary measures:

- Fluid intake should be between 2.5-3L/day or it should be sufficient to produce urine output of 2L/day
- Diet rich in fibres, fruits (orange, lemon, apple etc) & vegetables
- Maintain adequate calcium intake (1000-1200 mg/day)
- Aim to achieve/maintain healthy weight
- Reduce intake of animal protein (meat & dairy)

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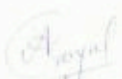
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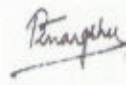
### Test Report

#### Specific measures based on metabolic evaluation and stone composition:

| STONE COMPONENT                                             | METABOLIC PROFILE               | SUGGESTED DIETARY RECOMMENDATION                                                                                                                                        |
|-------------------------------------------------------------|---------------------------------|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Calcium oxalate or combined calcium oxalate/phosphate stone | Hypercalciuria                  | Salt restriction                                                                                                                                                        |
|                                                             | Hyperoxaluria                   | Calcium supplements with meal<br>Restrict items rich in oxalate eg.<br>Soya protein, Spinach, Nuts,<br>Strawberries, Cocoa, Chocolates,<br>beetroot, carrot, radish etc |
| Uric Acid                                                   | Hyperuricosuria                 | • Limit animal protein & purine intake (Fish, lamb, beef, chicken, pork etc), cauliflower, Mushroom<br>• Reduce alcohol intake                                          |
|                                                             | Hypocitraturia                  | Increase citrate rich food (Orange, lemon, apple etc)                                                                                                                   |
|                                                             | Hyperuricosuria & Hyperuricemia | • Limit animal protein & purine intake (Fish, lamb, beef, chicken, pork etc), cauliflower, Mushroom<br>• Reduce alcohol intake                                          |
| Cystine                                                     | Metabolic syndrome              | Lifestyle changes, weight loss, Diabetes control                                                                                                                        |
|                                                             |                                 | • Increase fluid intake to 3-4L, Urine output should at least be 3L<br>• Restrict sodium & protein intake                                                               |

  
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End of report

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